

COVERAGE & CONTINUITY

Extended Leave Coverage Plan & Clinical Handoff Pack

A 12-week runway for parental, medical or sabbatical leave: caseload triage, patient letters, handoff summaries and a return checklist.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Plan + 4 letter templates

VERSION

1.0 · Sept 2026

TIME TO ADAPT

1-2 hours

WHAT IT IS

A step-by-step plan and template set for stepping away from practice for more than two weeks without stranding patients.

USE IT WHEN

Planned leave over 14 days: parental leave, medical leave, sabbatical, relocation or phased retirement.

CUSTOMIZE FIRST

Your state's notice and records rules, payer notification requirements, and whether your covering colleagues are licensed where your patients are located.

The runway at a glance

Twelve weeks is the comfortable version. If you have less time, keep the order and compress the steps. If leave is sudden, start at the one-week row and use the Reciprocal Coverage Agreement (PA-01).

When	Do this
12 weeks before	Set leave dates. Tell your professional liability carrier. Identify covering colleague(s). Check whether your lease, EHR and telehealth contracts need notice. Check payer requirements: in-network clinicians may have to notify the plan or arrange a credentialed substitute.
10 weeks	Triage the caseload (Step 1). Update your informed consent to name the covering colleague. Start conversations with patients who will transition.
8 weeks	Send the leave notice (Template 1) or referral-out letter (Template 2). Book transition or warm-handoff sessions. Prepare a Coverage Summary for every patient staying in care.
4 weeks	Confirm coverage in writing (PA-01). Test your auto-reply and after-hours message. Arrange secure record access for the covering colleague if needed. Prescribers: finalize the refill and bridging plan.
1 week	Final risk review of high-risk patients. Warm-handoff calls. Set voicemail and auto-reply (Template 3). Make sure every patient has crisis numbers.
During leave	Covering colleague documents every contact. Agree a short weekly check-in with them.
2 weeks before return	Send the return notice (Template 4). Review coverage notes. Schedule the debrief. Phase the caseload back in.

Step 1: Triage the caseload

Sort every patient into one of four tracks and document the track and the reason in the record.

Track	Fits patients who...	Action
A. Pause and return	Are stable or near maintenance, or can reasonably wait.	Notify. Give the covering colleague's name and crisis resources. Resume on return.
B. Covered continuity	Need occasional contact or have moderate risk, and benefit from staying with you.	Covering colleague handles urgent contact and, if agreed, periodic check-ins. You resume on return.
C. Temporary transfer	Need weekly clinical work during your absence.	Warm handoff to a covering clinician for the duration, with clear terms for return. The patient may choose to stay with the new clinician.
D. Refer out	Face a long leave, need more than you can guarantee on return, or would be better served elsewhere.	Referral-out with warm handoff and records transfer (see PA-07).

HIGH-RISK PATIENTS

Anyone with recent suicidal behavior, unstable psychosis or mania, severe substance use or active safety concerns should not receive a notice letter alone. Offer a joint session or a documented warm-handoff call between clinicians, confirm a current safety plan is in place, and confirm the patient holds crisis numbers.

PRESCRIBERS: DO NOT LEAVE PATIENTS WITHOUT MEDICATION ACCESS

Write prescriptions with enough supply plus a buffer, but remember that controlled-substance refill rules, state PDMP checks and telemedicine rules limit what can be pre-written. Identify a prescribing colleague who will manage refills and urgent medication questions. "Coverage" that leaves patients unable to obtain a refill is not coverage.

Step 2: Patient communications

Never put the reason for your leave in writing unless you choose to. Keep clinical detail out of auto-replies and voicemail.

Template 1: Leave notice (coverage retained)

Dear [Patient first name],

I am writing to let you know I will be away from my practice from [start date] until [return date]. I want your care to continue safely while I am away, so here is what to expect.

Sessions: [Your sessions on (dates) will be rescheduled / held with Dr. (Covering clinician) / paused until my return].

If you need help while I am away: my colleague Dr. [Name] is available for urgent needs at [phone / secure contact]. Dr. [Name] will have the information needed to help you, as described in the consent form you signed.

In an emergency call 911. For an emotional crisis, call or text 988, or chat at 988lifeline.org.

I will contact you on [date] to talk through the plan, or you are welcome to call sooner. Changes like this can stir up feelings, and we can talk about that.

Sincerely, [Your name and credentials]

Template 2: Referral-out and transition letter

Dear [Patient first name],

I am writing about your care during and after my leave from [start date] to [return date]. Because of the length of my absence [and because of what I think will serve your treatment best], I recommend that you continue with Dr. [Name], who [brief, respectful reason for the match, such as experience with your concerns].

With your written permission I would like to share a short summary of our work so you do not have to start over. I would also like to offer a joint session or call on [date] to introduce you. You are free to choose another clinician, and I am glad to give you other names.

[Optional: When I return you are welcome to resume with me if you wish, subject to my availability, or to stay with Dr. (Name).]

In an emergency call 911. For an emotional crisis, call or text 988. Thank you for the work we have done together.

Sincerely, [Your name and credentials]

Template 3: Voicemail and auto-reply

You have reached the office of [Name, credentials]. I am away from [date] to [date]. For urgent clinical needs, please contact Dr. [Name] at [phone]. If this is an emergency, call 911. If you are in emotional crisis, call or text 988. I will return non-urgent messages after [date].

Template 4: Return notice

Dear [Patient first name], I am pleased to let you know I will return on [date]. Our next session is scheduled for [date and time]. [If you worked with Dr. (Name) while I was away, I have reviewed the contact notes so we can pick up where things stand.] If you would like to talk about how you would like to proceed, please call me at [phone].

Step 3: Coverage summary and clinical handoff

Use Exhibit A of the Reciprocal Coverage Agreement (PA-01) for each patient in tracks B and C, and add the fields below for anyone who will be seen in session by another clinician.

Patient name or ID

Track (A / B / C / D)

Handoff completed on

Treatment goals and progress to date

What is working, and what to avoid

Patient's reaction to the leave and what they have said they need

Measures and current scores (name, date, score)

Decision points while I am away (what should prompt a change in level of care)

Next scheduled contact

Who owns it

Step 4: Return checklist

- ☐ Review every coverage note and contact log before your first session back.
- ☐ Contact each track B and C patient within **[5]** business days and reassess risk.
- ☐ Update treatment plans that changed during coverage.
- ☐ Confirm each patient's preference: continue with you, stay with the covering clinician, or transition.
- ☐ Reconcile billing for any services delivered by covering clinicians.
- ☐ Thank and debrief covering clinicians. Ask what you would change next time.
- ☐ Restore voicemail and auto-reply. Notify payers and referral partners you are back.
- ☐ Update your professional will and coverage agreement if anything changed (PA-01, PA-03).

IN PSYALLIANCE

The Planner turns this pack into a workflow. Import your caseload from CSV or Excel, set your leave dates, and it ranks colleague matches, drafts outreach, and moves to the next candidate automatically if someone declines.

Sources and further reading

- APA Ethics Code, Standards 3.12 and 10.09 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- HHS OCR, HIPAA and disclosures for treatment purposes ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- DEA and HHS, Fourth Temporary Extension of COVID-19 Telemedicine Flexibilities for Prescription of Controlled Medications (Federal Register, 31 Dec 2025).

ABOUT THIS DOCUMENT

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